

case b

CASE B

As of May 29th, Derfel, named after great giant of Wales. Wool-Worker

Patient & His Life History

The patient is Derfel, born in Southwark in the year of our Lord 1594, at the loom since his eighth year — three decades of conditions most injurious to any constitution, let alone a choleric one. He has worked all this time beside the river and the dye-houses, whose putrid fumes, breathed without relief since boyhood, cannot but corrupt the humours of any man so subjected. His sign is Aries, his ruling planet Mars: the hot and dry governor of choler. The Diagram shows that Aries falls in spring, hot and moist, governed by blood. But, in Derfel, the martial influence has quite overwhelmed the sanguine season, producing an environment in which choler predominates violently.

His diet compounds all of this. Salt beef, onions, small beer, little fresh provision is exactly the diet the Compendium names as the origin of scorbutic and leprous corruption (Compendium, pg. 13). This past summer he laboured carrying wool-packs in great heat *after* the floods, when the Thames banks were yet producing the rotting vapours that inundations generate (Compendium, pg. 44). He then ate unripe green plums fresh from that labour. They were cold, obstructive crude matter introduced into a belly already suffering.

Symptoms & Their Significance

Derfel cannot keep bread or meat for an hour in his stomach. He purges twelve times or more daily, sometimes nothing but blood. Cold shivers follow each purging while the interior fire persists. His pulse is worming (?) and fluttering and his vital spirits are taxed and failing. His body is lean and dry throughout.

His urine is orange. The Urine Wheel places orange or saffron urine among those signifying excessive digestion: the humours overcooked, burned past their due concoction (Ketham, Urine Wheel) where choleric excess is made visible. The bloody flux is the gravest sign: the Compendium warns that as dysentery advances, pure blood is passed and the bowels face incurable gangrene (Compendium, pg. 7).

Diagnosis & Its Rationale

Derfel has a bilious dysentery. Diarrhoea passes without blood or bowel inflammation, while dysentery does not (Compendium, pg. 7). Derfel's bloody flux settles the matter. I considered bilious colic, but the Compendium specifies that colic urine is thick from the outset

(Compendium, pg. 9) — Derfel's is orange, not thick, pointing firmly to the choleric dysentery. The fluttering pulse and the bowel's violence confirm this diagnosis.

Etiology

Derfel's Mars-ruled constitution predisposes him to hot and dry excess. Thirty years of dye-house fumes and river stink have loaded the blood with putrid particles. Salt beef and small beer have piled saline corruption upon choleric corruption. Summer labour in post-flood heat delivered the final insult of rotting Thames vapours (Compendium, pg. 44). The unripe plums then struck a belly already inflamed. This is the exigence of his visit to me. The bowels expel without cease and the blood, driven outward by excess choler, appears in the stool.

Prescription

We must bloodlet! We will bleed from the arm because the the choleric blood is too turgid (Compendium, pg. 7). A paregoric that same evening. Next morning, the common purgative potion, repeated on alternate days twice more, paregoric after each. On days without purging, paregoric morning and evening. For drinking he should have boiled milk with three parts water, or white decoction of calcined hartshorn and bread-crumbs, sweetened with white sugar (Compendium, pg. 7-8). Barley water with cinnamon and prepared pearls as a julep when weak. After the second or third purging, laudanum twice or thrice daily to arrest the flux (Compendium, pg. 8). The salt diet must be abandoned entirely, and the dye-house quarter quit if it can at all be managed.

Prognosis

Recovery from his crisis is possible if the bloody stools have not yet advanced to pure blood and gangrene. But Derfel's constitution has been bound for failure since boyhood, the fluttering pulse already signals failing strength, and the conditions that made him sick remain his conditions of life. If he survives without a change in how he lives, the disease will return. He must quit salt meats and the dye-house air, or there is little more I can do for him.

Personal Addendum

Case B was the easiest to diagnose for me — the symptoms mapped nicely onto the Compendium's dysentery entry, and the cholera was identifiable from the Humors diagram. The urine wheel was especially useful in ruling out bilious colic. I am pretty confident in the dysentery diagnosis, but obviously I'm a bit uncertain how heavily a period physician would have weighted the miasmatic and environmental history/causes relative to the humoral constitution of the patient. I had so much fun watching the video though.

case c

CASE C

As of May 29th, Pyotr Makepeace, Traveller

Patient & His Life History

Pyotr Makepeace, born in Anglia in the year of our Lord 1567 beneath the sign of Gemini, and now some fifty-eight or fifty-nine years of age. Gemini is hot and moist, the province of Air and the sanguine humour (Diagram of Humors, sanguine quadrant). The sanguine temperament fits this man well since he has chased dancing, poems, and parties, travelling from France to Spain to Italy in pursuit of good company. Ordinarily, the most healthful of the four, but Pyotr is no longer young. The Compendium is clear that pestilential fevers strike harder when persons are well grown in years, and reserves its hopeful prognosis for youth, sound flesh, and sanguine temperament together (Compendium, pg. 24). Age has quietly undone the advantage his constitution used to give him.

He now lives in what he calls the Darklands, which are foggy, cloudy, damp, the parties endless, the air close and crowded. This is a man who is breathing corrupted air day and night without relief. The Compendium names marshy grounds, close air in houses, stinking accumulations near dwellings among the chief causes of plague (Compendium, pg. 26, 45). Furthermore, Pyotr himself reports strange and unseasonable weather during his recent travels through France, Spain, and Italy. The Compendium explicitly names a year that does not keep its temperament, with immoderate excesses of heat, cold, drought, or moisture, as a direct presage of plague (Compendium, pg. 26). The patient's own testimony is evidence.

His diet has been merrily intemperate with meat, olives, wine in abundance. Still, fear has suppressed his appetite.

Symptoms & Their Significance

Pyotr presents with alternating chills and burning heat, pounding in the head, and intense thirst during the hot phase, the cycle then repeating. A fear overtakes him and with every fit and chills bring strange dreams like blood dripping down the stairs of a chapel. His appetite has also gone.

Urine is clear. On the Urine Wheel, clear urine like spring water signifies troubled health (Ketham, Urine Wheel) — the body's concocting warmth consumed by the fever's violence, leaving nothing for ordinary digestion.

The alternating chills and fever match the Compendium's plague description exactly: incalcescence, lassitude, thirst, burning of the praecordia, sudden fall of strength

(Compendium, pg. 26). The fear is not merely a symptom but an active worsening agent — the Compendium warns that fear and sadness draw the pestilential seeds inward toward the heart (Compendium, pg. 28), and further that those who dread the disease fall into it more readily (Compendium, pg. 36). Pyotr suspects plague and fears it intensely. His dread is feeding the very thing he dreads.

Diagnosis & Its Rationale

I diagnose Pyotr with a pestilential fever, with strong suspicion of true plague. The alternating heats and chills, the clear urine, the bloody fearful dreams, and the history of disordered-weather travel followed by permanent settlement in damp close quarters all support it. Buboes and carbuncles have not yet appeared, which leaves the diagnosis not entirely certain — but does not overturn it.

The English sweat was considered. The Compendium describes it as a fever in the spirits with head pain, heaviness, and a marvellous desire to sleep (Compendium, pg. 42). Consistent enough to warrant consideration. But the sweat resolves, for good or ill, within twenty-four hours, fire in reeds, soon in flame and soon out (Compendium, pg. 43). Pyotr's symptoms persist so duration rules the sweats out.

Etiology

The miasm was most likely first received during Pyotr's travels through regions of strange, disordered weather — a plague-promoting year by the Compendium's own signs. Settling thereafter in the Darklands has made that exposure permanent with damp, foggy, close air delivering fresh miasm to his lungs and spirits. His sanguine blood has received the fermentative venom readily. His crowded nightly parties in close foggy rooms have ensured the contagion has had every opportunity to take hold. His age means the animal spirits can no longer resist or expel the venom. And the fear that grips him at every fit actively drives the venom inward from the body's circumference toward the vital parts (Compendium, pg. 28). Every element of his present life is working against him.

Prescription

Treatment must begin now. *“God's help being implored, we must presently fly to Remedies”* (Compendium, pg. 30). First, a vomit to clear the passages. Then diaphoretics without delay, to drive the venom outward by sweat, away from the heart (Compendium, pg. 30). Alexipharmics should be given continuously. Pyotr must leave the Darklands to somewhere open, dry, and well-ventilated. Above all his fear must be fought because *A cheerful and confident Mind is better than the most exquisite Antidote* (Compendium, pg. 28).

Prognosis

Doubtful... The Compendium's hopeful prognosis belongs to youth, robust body, and fortitude of mind (Compendium, pg. 27) — Pyotr has none of these in full. He is nearly sixty, his spirits already disordered in dreams and fear, his appetite gone. The one encouraging sign is the absence of buboes and carbuncles, suggesting the venom has not yet fully coagulated the blood. If the diaphoretics expel it before that occurs and the patient's mind can be steadied, he can recover.

Personal Addendum

The symptom of alternating chills and fever fits both the sweating sickness and plague, and I had to work hard to distinguish between them using the Compendium's descriptions. I settled on plague partly because of the travel history through damp regions and the intermittent (rather than purely swift) nature of the disease. The role of fear in plague etiology, as the Compendium describes it, struck me as very psychologically rich. I found it challenging to link the strange dreams to a humoral mechanism. I am moderately confident in this diagnosis, but the sweats remain a plausible alternative.

case e

CASE E

As of May 29th, Chad from Chadwick

Patient & His Life History

The patient gives his name as Chad, from the district of Chadwick, his natal sign Pisces. Pisces falls cold and moist, the province of Water and the phlegmatic humour (Diagram of Humors, phlegmatic quadrant). The phlegmatic man is slow, heavy, sluggish, much afflicted by cold, his stomach full and swollen and Chad fits the description well: perpetually tired in a way unlike himself, stiff especially in the cold, heavy in his movements. His life is exactly the conditions our sources name as causes of leprous and scorbutic corruption. Born near a swamp in Butorn, he kept sheep and pigeons in the marshes as a youth, then travelled and fraternized. In each place he exchanges one waterlogged miasmatic environment for another. The Compendium says that scurvy and kindred leprous conditions are endemic to maritime and marshy places, and that corrupted water and damp air are principal causes (Compendium, pg. 32).

His diet throughout has been almost entirely fish — salt fish, stewed, grilled, shellfish — and here the patient supplied a detail without apparent concern that the physician cannot overlook: the fish goes bad easily. The Compendium holds that corrupt foods introduce into the blood particles that cannot enter due mixture with it and cannot be mastered by the natural heat (Compendium, pg. 13). A man who has eaten habitually spoiling fish for decades has been loading his blood with the ordinary unwholesomeness of salt and shellfish and with actively corrupt matter.

Symptoms & Their Significance

Chad presents with stiffness of the joints in cold weather; fatigue unlike his usual habit; heat around the middle; difficulty walking; itching skin; dark spots and marks across the body for some ten weeks; a swollen tongue with small white spots; and a spreading numbness over the body. The urine is cloudy and murky with specks and sediment. The Urine Wheel places such urine among those signifying troubled health (Ketham, Urine Wheel) although I was unsure of which dial on the wheel exactly fit this urine's presentation, and the Compendium notes specifically that lepers' urine may be oily and full of threads, the disease's heat corrupting the oil within it (Compendium, pg. 12). Chad's urine could accord with this.

The pin test requires a careful note. The Compendium's prescribed method is to cover the patient's eyes, *threaten* to prick but withhold the prick, then ask if he felt it — the leper, his sensation disordered, will say yes to a prick that never came (Compendium, pg. 12). In Chad's case the physician actually applied the pin to the left hand (I think). Chad correctly named the

location but said it was not very painful and that he does not feel much. This is not the classical test as written.... and thus the results from this test are inconclusive. Still the finding is potentially genuine diminishment of real sensation. It supports the diagnosis by the same underlying logic, even if it arrived there differently. The swollen tongue with white spots is consistent with the Compendium's observation that lepers' tongues are dry and their blood and voices corrupted (Compendium, pg. 12).

Diagnosis & Its Rationale

I diagnose Chad with leprosy. The running scab or leprosy of the Greeks, which the Compendium describes as beginning with little red or blue wheals or bruises joined together, the surface growing rough, whitish, and scaly (Compendium, pg. 12). The dark marks, the itching, the diminished sensation, the cloudy particulate urine, and the swollen spotted tongue together satisfy the Compendium's five signs of leprosy: urine, pulse, blood, voice, and skin (Compendium, pg. 12). Scurvy was considered. The Compendium explicitly links the two, noting the running scab often succeeds an inveterate scurvy (Compendium, pg. 15), and Chad's spots, weak limbs, aching joints, and muscular stupor are real scorbutic signs (Compendium, pg. 31-32). The insensibility, the tongue, and the character of the lesions tip the balance to leprosy as primary.

Etiology

Born under Pisces into a cold watery constitution, Chad was from the outset poorly equipped to resist damp, cold, and unwholesome diet. The swamps of Butorn corrupted his blood from childhood with miasmatic exhalations and foul water. His subsequent travels to the river delta, then north Scottish marshes renewed and deepened it at each stage. Throughout, his diet was predominantly fish, including shellfish the Compendium names as a direct cause of the running scab (Compendium, pg. 12-13), and fish that by his own admission frequently went bad. Putrefied matter, introduced year after year into a phlegmatic blood least able to master it, accumulated until the crasis was so far corrupted that the tainted blood began depositing itself in the skin (Compendium, pg. 13). The nervous juice, drawing upon that depauperated blood, has been corrupted in turn — hence the spreading numbness (Compendium, pg. 32-33).

Prescription

Remove the causes first, before all else (Compendium, pg. 13). Chad must quit shellfish, salt fish, and above all any fish he suspects of spoiling. He must leave marshy habitation immediately for dry open air: the Compendium is explicit that no cure takes hold while the patient remains in the conditions that made him sick (Compendium, pg. 13).

For the cure: general purge and bleeding, then the cathartic infusion of sharp-pointed dock, polypody, sena, and rhubarb in white wine, six to eight ounces, repeated within six or seven

days (Compendium, pg. 14). Daily whey — two to three pounds each morning — with fumitory, chicory, and sharp-pointed dock infused within it, for twenty or thirty days (Compendium, pg. 14). Mineral waters from iron springs to be added, these having succeeded even where other remedies failed (Compendium, pg. 14).

Prognosis

The running scab confirmed and drawing near to leprosy is hardly or never cured (Compendium, pg. 15). Ten weeks of visible eruption, spreading numbness, decades of accumulated corruption are not favourable conditions. Some cures by whey and mineral waters are recorded even in advanced cases, and Chad retains something of his strength, so we need not utterly despair. But, a complete cure is unlikely but if diet and habitation are wholly changed and the prescribed course pursued for many months without interruption, the disease's progress may be slowed.

Personal addendum.

Case E was easier diagnostically, because the leprosy section of the Compendium contained the pin-prick test, even though I was unsure of how the pin prick test was received, this hinted me heavily towards leprosy. The match between the described signs of leprosy (urine, tongue, skin, insensibility) and the case notes also gave me grounds for confidence it was leprosy. Still found it challenging to decide whether this was primarily a leprous or a scorbutic case—the Compendium explicitly links the two, which complicated things. I ultimately made leprosy primary and any other things, like scurvy, a contributing illness. The phlegmatic constitution under Pisces in a marshy environment felt pretty coherent as an etiological framework.

Tab 1

Instructions FINISH BY THURSDAY NIGHT

3. Please choose any three of the cases (the video files called “Case A” through “Case E”) and write case histories for each, as considered through the cosmology and reasoning of an early modern physician.

MY NOTES NOTES ON EACH VIDEO FILES:

CASE B

Derfel

Weak in belly
Greavis ring in guts
Cant keep bread or meat for an hour in stomach
Pulse: worming fluttering
Lean and dry
Aries, mars
Loom work
Close by river,
Labored in great heat
Wool packs
Diet = salt beef onions small beer
Little fresh stuff
Urine is orange
Belly burns like coal
But Cold shivers after purging
Purged 12 times yesterday, sometimes blood comes out
Pain makes him like a savage
Ateunripe green plums after laboring in the heat

GO BACK TO LOOK AT THEIR LIFE HISTORIES

CASE C

Pyotr

Chills and pounding in head
Burning heat and thirst
Then hcills
Fear when heat and chills come
Strange dreams accompany when chills come - blood on stairs of capel
Anglia, Beneath gemini
Fancied dancing nad poems and parties
Travel fara nd wide for party - france to spain then to italy
Dark lands parties? Foggy cloudy and DAMP

Diet: meat olives and wine, but now sick with fear not as hungry
Urine is clear
keep s sweating and outching face
Patient thinks they are signs of plague

GO BACK TO LOOK AT THEIR LIFE HISTORIES

CASE E:

Joints stiff in cold weather
Tired all the time, not usual
Heat around the middle
Difficult to walk
Skin itchy
Darkk spots and marks all over his body - had for about 10 weeks
Born near a weeden swamp
Pisces
Sheep and pigeons in teh marshes
Went to river as a kid
Scotland etc
Stoned? Grilled fish? Shellfish?
When he opens his mouth, tongue swollen, little white spots
Stuck him with a pin on the left hand, - doesnt feel much but “kinda painful”
URINE: cloudy, a little yellow, but al little clear why are there specks in it
Numbness spreads all over the body

GO BACK TO LOOK AT THEIR LIFE HISTORIES

4. Please title your case histories so they correspond to the numbers of the cases. In other words, please make sure that if you’re writing about Case E, your document also indicates Case E.

5. Your case history should address questions such as:

Who is the patient? What can you tell from their life history, their behavior, their carriage, etc. How is this information medically relevant?

What are their symptoms? Which ones are significant, and why?

What’s your diagnosis? Did you consider any alternate diagnoses? If so, why?

What’s the etiology of the disease (that is, how does the disease “work” in this particular patient; why is the patient sick?)

What’s your prescription, and why?

What’s your prognosis, and why?

6. Your case histories can be modeled on the sample case history found in the document entitled “ModelCaseHistory.pdf” but they need not hew exactly to

that format.

7. Case histories should be between 500 and 1000 words. Please do not exceed 1000 Words.

8. Feel free to use contemporary English in writing your case histories. If you wish to try writing in the manner of an early modern doctor, go for it.

9. The document entitled “Compendium of Physick” contains distillations and amalgamations of the case notes of several authors writing about different diseases. You can use these to make your diagnoses, prognoses, and Prescriptions.

10. In the same folder that you found the “Compendium of Physick” you will also find in the a diagram of the humors in relation to elements, time of year, stage of life (etc.); and a uroscopy wheel. You can also use these in your diagnoses.

11. Please cite these documents in your case notes. For instance, if page 9 of the “Compendium” tells you that a hot and dry complexion disposes a patient to quartan fever, and you use this information in your case notes, please cite “Compendium, pg. 9” (or similar). Of course, cite direct quotes if you use them.

12. Your goal is to render a plausible, compelling diagnosis reasoned in the manner of an early modern scholarly physician.

13. Finally, please include a very short personal addendum at the end of your case study. In this addendum, please tell me how you think you did (honestly!), which parts you found challenging, which parts you found straightforward, etc. I won't take your addendum into account when grading, except insofar as it helps me better understand how you approached the assignment.